NUR 774 Cardiac Addendum: 50 Practice Questions

19 Select All That Apply + 31 single answer, grouped by topic. Each question is followed by its answer, rationale, and a mnemonic.

> Heads-up: the addendum's Kawasaki slide says ibuprofen; your integrated study guide and current AHA guidance say ASPIRIN + IVIG. Ask your professor which they want.

How to use: answer the question, then swipe to the next page for the answer, rationale, and mnemonic.

SINGLE ANSWER

Which history finding in a 2-month-old most suggests a heart problem?

A. Sleeping about 16 hours a day

B. Sweating and tiring during feedings

C. Spitting up small amounts after feeds

D. Six wet diapers a day

Swipe for answer →

Answer 1

B

- **Sweating and tiring during feedings**

Why: Feeding is hard work for an infant. Diaphoresis, fatigue, and long feeds signal heart failure. The others are normal infant findings.

> 🧠 **"Feeding is the infant's treadmill." Sweaty, tired feeder = check the heart.**

SELECT ALL THAT APPLY

Which tests are used to assess heart disorders in children?

A. Exercise testing

B. Lumbar puncture

C. Chest radiography

D. Cardiac MRI

E. Electrocardiogram (ECG)

F. Echocardiography

G. Tympanometry

Swipe for answer →

Answer 2

A, C, D, E, F

- **Exercise testing**
- **Chest radiography**
- **Cardiac MRI**
- **Electrocardiogram (ECG)**
- **Echocardiography**

Why: The addendum lists ECG, radiography, echocardiography, phonocardiography/MRI, exercise testing, and labs.

> 🧠 **"Every Child's Heart Exam Matters": ECG, Chest x-ray, Heart echo, Exercise test, MRI.**

SINGLE ANSWER

Which test best shows a hole between the heart chambers?

A. Pulse oximetry

B. Echocardiogram

C. Phonocardiography

D. Electrocardiogram

Swipe for answer →

Answer 3

B

- **Echocardiogram**

Why: Echo shows anatomy, valves, and blood flow direction. An ECG shows electrical activity only.

> 🧠 **"Echo = Eyes (sees the anatomy). ECG = Electricity."**

SELECT ALL THAT APPLY

A nurse is doing a physical assessment on a child with a suspected heart defect. Which should be included?

A. Visual acuity

B. Respiratory rate and effort

C. Tympanic membrane mobility

D. General appearance and skin color

E. Pulses in the arms AND legs

F. Blood pressure

Swipe for answer →

Answer 4

B, D, E, F

- **Respiratory rate and effort**
- **General appearance and skin color**
- **Pulses in the arms AND legs**
- **Blood pressure**

Why: Cardiac physical assessment = general appearance (color, cyanosis, growth), pulses, BP, and respirations.

> 🧠 **"Look, Feel, Measure": Look (color, appearance), Feel (pulses in all 4 limbs), Measure (BP, HR, RR).**

SINGLE ANSWER

Symptoms of a large VSD usually appear at what age?

A. 6 months

B. 4–8 weeks

C. The first hour of life

D. School age

Swipe for answer →

Answer 5

B

- **4–8 weeks**

Why: Pulmonary resistance falls over the first weeks, so more blood shunts left → right and the infant becomes symptomatic around 4–8 weeks.

> 🧠 **"4 to 8, the lungs relax and the VSD floods them."**

SINGLE ANSWER

Which murmur is classic for a VSD?

A. Fixed split S2

B. Continuous machinery murmur

C. Harsh holosystolic murmur at the lower left sternal border

D. Soft musical murmur that is louder lying down

Swipe for answer →

Answer 6

C

- **Harsh holosystolic murmur at the lower left sternal border**

Why: The addendum says "fatigue and harsh murmur." Machinery = PDA; fixed split = ASD; soft musical = Still's (innocent).

> 🧠 **"VSD = Very loud." Small hole, loud howl.**

SELECT ALL THAT APPLY

Which statements about a ventricular septal defect are correct?

A. It increases pulmonary blood flow

B. It always needs emergency prostaglandin E1

C. Many small VSDs close on their own

D. VSDs that do not close may need surgery

E. It is an opening between the ventricles

F. It causes cyanosis at birth from right-to-left flow

Swipe for answer →

Answer 7

A, C, D, E

- **It increases pulmonary blood flow**
- **Many small VSDs close on their own**
- **VSDs that do not close may need surgery**
- **It is an opening between the ventricles**

Why: A VSD is a left-to-right (acyanotic) shunt. Most small ones close spontaneously; persistent significant ones need surgical closure.

> 🧠 **"Small hole: wait and watch. Big hole: fix it."**

SINGLE ANSWER

A parent asks if their infant's small VSD will need surgery. Which response is best?

A. "VSDs never close without surgery."

B. "The loud murmur means it is a large, dangerous hole."

C. "All VSDs need open-heart surgery right away."

D. "Many small VSDs close on their own; the cardiologist will follow it."

Swipe for answer →

Answer 8

D

- **"Many small VSDs close on their own; the cardiologist will follow it."**

Why: Most small VSDs close spontaneously. A small VSD can be LOUD; loudness does not equal severity.

> 🧠 **"Small hole, loud howl, often closes."**

SINGLE ANSWER

Which heart sound is the classic sign of an atrial septal defect?

A. Continuous machinery murmur

B. Fixed splitting of the second heart sound (S2)

C. Harsh holosystolic murmur

D. Diastolic murmur at the apex

Swipe for answer →

Answer 9

B

- **Fixed splitting of the second heart sound (S2)**

Why: Extra right-sided flow delays pulmonic valve closure, so S2 is widely split and doesn't change with breathing.

> 🧠 **"ASD = Always Split, Doesn't change."**

SELECT ALL THAT APPLY

Which defects INCREASE pulmonary blood flow?

A. Tetralogy of Fallot

B. Ventricular septal defect

C. Atrial septal defect

D. Pulmonic stenosis

E. Tricuspid atresia

F. Patent ductus arteriosus

Swipe for answer →

Answer 10

B, C, F

- **Ventricular septal defect**
- **Atrial septal defect**
- **Patent ductus arteriosus**

Why: Left-to-right shunts send extra blood to the lungs. TOF and tricuspid atresia DECREASE pulmonary flow; PS is obstructive.

> 🧠 **"The D's flood the lungs": ASD, VSD, PDA all end in D.**

SINGLE ANSWER

The ductus arteriosus connects which two structures?

A. Pulmonary artery and aorta

B. Right and left ventricles

C. Umbilical vein and inferior vena cava

D. Right and left atria

Swipe for answer →

Answer 11

A

- **Pulmonary artery and aorta**

Why: In the fetus it lets blood skip the lungs. It should start closing with the first breaths (the addendum says closure is complete by about 3 months).

> 🧠 **"Ductus ARTERIOSUS connects the two big ARTERIES."**

SELECT ALL THAT APPLY

Which findings are expected with a significant PDA?

A. Continuous machinery murmur

B. Wide pulse pressure

C. Weak femoral pulses with high arm blood pressure

D. Cyanosis at birth

E. Bounding pulses

F. Fixed split S2

Swipe for answer →

Answer 12

A, B, E

- **Continuous machinery murmur**
- **Wide pulse pressure**
- **Bounding pulses**

Why: Blood runs aorta → pulmonary artery during systole AND diastole, which makes the continuous murmur and wide pulse pressure.

> 🧠 **"PDA = Machinery, Wide, Bounding" ("Mighty Wide Bounce").**

SINGLE ANSWER

A premature infant has a symptomatic PDA. Which medication does the nurse anticipate?

A. Digoxin

B. Prostaglandin E1

C. Indomethacin

D. Furosemide only

Swipe for answer →

Answer 13

C

- **Indomethacin**

Why: Indomethacin (or ibuprofen) blocks prostaglandin and helps the ductus close in preemies.

> 🧠 **"Indomethacin Interrupts (closes) it. Prostaglandin keeps it Patent (open)."**

SELECT ALL THAT APPLY

Which are management options for a PDA?

A. Indomethacin in selected premature infants

B. Antibiotics to dissolve it

C. Prostaglandin E1 to close it

D. Closure during cardiac catheterization

E. Surgical ligation

Swipe for answer →

Answer 14

A, D, E

- **Indomethacin in selected premature infants**
- **Closure during cardiac catheterization**
- **Surgical ligation**

Why: The addendum lists indomethacin, cardiac catheterization, and surgery. PGE1 does the OPPOSITE: it keeps the ductus open.

> 🧠 **"Pill, Plug, or Procedure": Indomethacin (pill), cath device (plug), surgery (procedure).**

SINGLE ANSWER

Pulmonic stenosis causes hypertrophy of which chamber?

A. Right ventricle

B. Left atrium

C. Left ventricle

D. Right atrium only

Swipe for answer →

Answer 15

A

- **Right ventricle**

Why: The right ventricle must push blood through a narrow pulmonary valve, so it thickens.

> 🧠 **"Pulmonic = Pushes the Right side."**

SELECT ALL THAT APPLY

Which findings are associated with pulmonic stenosis?

A. Signs of right-sided heart failure

B. Systolic ejection murmur

C. Cyanosis when severe

D. Right ventricular hypertrophy

E. Bounding pulses

F. Continuous machinery murmur

Swipe for answer →

Answer 16

A, B, C, D

- **Signs of right-sided heart failure**
- **Systolic ejection murmur**
- **Cyanosis when severe**
- **Right ventricular hypertrophy**

Why: The addendum: mild right-sided heart failure, cyanosis, systolic ejection murmur (SEM). Critical PS can limit lung blood flow, causing cyanosis.

> 🧠 **"PS = Pressure on the Right, Systolic murmur."**

SINGLE ANSWER

What is the usual treatment to relieve pulmonic stenosis?

A. Bed rest only

B. Balloon angioplasty/valvuloplasty

C. Lifelong prostaglandin E1

D. Indomethacin

Swipe for answer →

Answer 17

B

- **Balloon angioplasty/valvuloplasty**

Why: A balloon catheter stretches the narrow valve open.

> 🧠 **"Narrow valve? Balloon it open."**

SINGLE ANSWER

Aortic stenosis leads to hypertrophy of which chamber?

A. Left atrium only

B. Right ventricle

C. Right atrium

D. Left ventricle

Swipe for answer →

Answer 18

D

- **Left ventricle**

Why: The left ventricle must push blood through a narrow aortic valve.

> 🧠 **"Aortic = Left (A comes Last, on the Left side)." Pulmonic = Right.**

SELECT ALL THAT APPLY

Which are concerns in a child with aortic stenosis?

A. Continuous machinery murmur

B. High arm BP with weak femoral pulses

C. Fainting (syncope)

D. A heart murmur

E. Chest pain with exertion

F. Risk of sudden death

Swipe for answer →

Answer 19

C, D, E, F

- **Fainting (syncope)**
- **A heart murmur**
- **Chest pain with exertion**
- **Risk of sudden death**

Why: The addendum: usually no symptoms except a murmur, but chest pain and even sudden death can occur. Exertional symptoms are red flags.

> 🧠 **"AS = Avoid Sudden exertion": chest pain, syncope, sudden death.**

SINGLE ANSWER

A teen with known aortic stenosis wants to join the basketball team. Which response is best?

A. "Sports are fine since you have no symptoms."

B. "You need a cardiology evaluation before being cleared for sports."

C. "Only avoid sports if you get a fever."

D. "Just drink extra water during games."

Swipe for answer →

Answer 20

B

- **"You need a cardiology evaluation before being cleared for sports."**

Why: Exertion can cause chest pain, syncope, or sudden death in AS. Sports clearance must come from cardiology.

> 🧠 **"Faints while running = Referral."**

SELECT ALL THAT APPLY

Which are treatment options for aortic stenosis?

A. Prostaglandin E1 for long-term treatment

B. Valve replacement

C. Beta blocker or calcium channel blocker for stabilization

D. Indomethacin

E. Balloon valvuloplasty

Swipe for answer →

Answer 21

B, C, E

- **Valve replacement**
- **Beta blocker or calcium channel blocker for stabilization**
- **Balloon valvuloplasty**

Why: The addendum: stabilization with a beta blocker or CCB, balloon valvuloplasty, or valve replacement.

> 🧠 **"Calm it, Balloon it, or Replace it."**

SINGLE ANSWER

Which finding is classic for coarctation of the aorta?

A. Higher blood pressure in the arms than the legs

B. Continuous machinery murmur

C. Higher blood pressure in the legs than the arms

D. Fixed split S2

Swipe for answer →

Answer 22

A

- **Higher blood pressure in the arms than the legs**

Why: The narrowing blocks flow to the lower body: pressure is high above it and low below it, with weak or absent femoral pulses.

> 🧠 **"Strong upstairs, weak downstairs."**

SELECT ALL THAT APPLY

Which symptoms may an older child with coarctation of the aorta have?

A. Bounding femoral pulses

B. Leg pain with activity

C. Headache

D. Nosebleeds

E. Cyanosis of the lips at rest

F. Dizziness (vertigo)

Swipe for answer →

Answer 23

B, C, D, F

- **Leg pain with activity**
- **Headache**
- **Nosebleeds**
- **Dizziness (vertigo)**

Why: High pressure in the upper body causes headache, vertigo, and nosebleeds (even stroke); poor flow to the legs causes leg pain.

> 🧠 **"Head, Nose, Legs": high pressure up top (headache, nosebleed), low flow down below (leg pain).**

SINGLE ANSWER

What is the best screening action for coarctation?

A. Check oxygen saturation only

B. Check capillary refill in one hand

C. Listen for a fixed split S2

D. Compare pulses and BP in the arms and legs

Swipe for answer →

Answer 24

D

- **Compare pulses and BP in the arms and legs**

Why: Four-extremity pulse and BP comparison catches the arm-leg difference.

> 🧠 **"Check all 4 corners": both arms, both legs.**

SINGLE ANSWER

A 3-day-old with coarctation suddenly becomes pale, poorly perfused, and in shock. What is the most likely cause?

A. Dehydration from breastfeeding

B. The ductus arteriosus is closing

C. Normal newborn transition

D. The foramen ovale is opening

Swipe for answer →

Answer 25

B

- **The ductus arteriosus is closing**

Why: In severe neonatal CoA, the open ductus was supplying the lower body. When it closes, perfusion drops. PGE1 can reopen it.

> 🧠 **"Duct closes → baby crashes."**

SINGLE ANSWER

In tricuspid atresia, how does blood leave the right atrium?

A. Through the ductus venosus

B. Through the mitral valve into the right ventricle

C. Through the tricuspid valve into the right ventricle

D. Through the foramen ovale into the left atrium

Swipe for answer →

Answer 26

D

- **Through the foramen ovale into the left atrium**

Why: The tricuspid valve is closed, so blood must cross the foramen ovale, then reach the lungs through a PDA (or VSD).

> 🧠 **"Front door locked? Take the side door" (the foramen ovale).**

SELECT ALL THAT APPLY

An infant with tricuspid atresia is at risk when the fetal shunts close. Which signs would the nurse expect?

A. Cyanosis

B. Dyspnea

C. Tachycardia

D. Fixed split S2

E. Wide pulse pressure with bounding pulses

Swipe for answer →

Answer 27

A, B, C

- **Cyanosis**
- **Dyspnea**
- **Tachycardia**

Why: The addendum: when these structures close, cyanosis, tachycardia, and dyspnea occur.

> 🧠 **"Shunts shut → Blue, Breathless, and Beating fast."**

SINGLE ANSWER

An infant with tricuspid atresia is receiving an IV prostaglandin E1 infusion. What is the nursing priority?

A. Restrict all oral fluids

B. Check for hearing loss

C. Monitor for apnea

D. Monitor for hypertension

Swipe for answer →

Answer 28

C

- **Monitor for apnea**

Why: PGE1 keeps the ductus open until surgery, but it can cause apnea. Continuous monitoring is required.

> 🧠 **"PGE1 = Patent ductus, Pause in breathing."**

SELECT ALL THAT APPLY

Which are the four defects in Tetralogy of Fallot?

A. Ventricular septal defect

B. Coarctation of the aorta

C. Right ventricular hypertrophy

D. Atrial septal defect

E. Pulmonary stenosis

F. Overriding (dextroposed) aorta

Swipe for answer →

Answer 29

A, C, E, F

- **Ventricular septal defect**
- **Right ventricular hypertrophy**
- **Pulmonary stenosis**
- **Overriding (dextroposed) aorta**

Why: The addendum: PS, VSD, dextroposition of the aorta, and RV hypertrophy.

> 🧠 **"PROV": Pulmonary stenosis, RVH, Overriding aorta, VSD.**

SINGLE ANSWER

A child with TOF has polycythemia. The nurse understands this is:

A. The body making more red blood cells because of low oxygen

B. A sign of iron deficiency

C. Caused by too much oxygen

D. An infection

Swipe for answer →

Answer 30

A

- **The body making more red blood cells because of low oxygen**

Why: Chronic hypoxemia stimulates RBC production to carry more oxygen.

> 🧠 **"Low O2 → More Reds."**

SELECT ALL THAT APPLY

Which findings reflect chronic hypoxemia in uncorrected TOF?

A. Bounding pulses

B. Cyanosis

C. Polycythemia

D. High arm BP with weak femoral pulses

E. Growth restriction

F. Dyspnea

G. Clubbing of the fingers

Swipe for answer →

Answer 31

B, C, E, F, G

- **Cyanosis**
- **Polycythemia**
- **Growth restriction**
- **Dyspnea**
- **Clubbing of the fingers**

Why: The addendum's TOF signs: cyanosis, polycythemia, dyspnea, growth restriction, clubbing.

> 🧠 **"Blue, Clubbed, Thick blood, Small, and Short of breath."**

SINGLE ANSWER

An infant with TOF suddenly becomes very blue and breathes fast while crying. What should the nurse do FIRST?

A. Place the infant in a knee-chest position

B. Feed the infant to calm them

C. Give albuterol

D. Lay the infant flat on the back

Swipe for answer →

Answer 32

A

- **Place the infant in a knee-chest position**

Why: Knee-chest raises systemic resistance, which pushes more blood into the lungs. Then calm the infant, give O2, and get emergency help.

> 🧠 **"Tet spell? Tuck like a frog."**

SINGLE ANSWER

Why does a child with uncorrected TOF squat after playing?

A. Squatting closes the VSD

B. Squatting lowers blood pressure

C. Squatting increases systemic resistance and sends more blood to the lungs

D. Squatting rests tired leg muscles

Swipe for answer →

Answer 33

C

- **Squatting increases systemic resistance and sends more blood to the lungs**

Why: Squatting kinks the leg arteries, raising systemic resistance so less blood crosses the VSD and more goes to the lungs.

> 🧠 **"Squat = Squeeze the legs, Send blood to the lungs."**

SINGLE ANSWER

A child with TOF and polycythemia has vomiting and diarrhea. Why is this a concern?

A. Dehydration thickens the blood and raises the risk of clots and stroke

B. It is not a concern

C. It lowers the red blood cell count

D. It causes the VSD to close

Swipe for answer →

Answer 34

A

- **Dehydration thickens the blood and raises the risk of clots and stroke**

Why: Polycythemic blood is already thick. Dehydration makes it thicker, raising the risk of thrombosis or stroke. Keep the child hydrated.

> 🧠 **"Thick blood + dry = clot risk": keep them hydrated.**

SELECT ALL THAT APPLY

Which are signs of RIGHT-sided heart failure?

A. Hepatomegaly

B. Crackles in the lungs

C. Increased venous pressure

D. Dyspnea as the first sign

E. Edema/weight gain from fluid

Swipe for answer →

Answer 35

A, C, E

- **Hepatomegaly**
- **Increased venous pressure**
- **Edema/weight gain from fluid**

Why: Right-sided failure backs fluid up into the body (liver, veins). Left-sided failure backs up into the LUNGS (crackles, dyspnea).

> 🧠 **"Left = Lungs. Right = the Rest of the body" (liver).**

SINGLE ANSWER

Which finding indicates LEFT-sided heart failure?

A. Ascites

B. Jugular venous distention

C. Crackles (rales) in the lungs

D. Hepatomegaly

Swipe for answer →

Answer 36

C

- **Crackles (rales) in the lungs**

Why: Left-sided failure → fluid backs up into the lungs: dyspnea, crackles, cyanosis. It eventually causes right-sided failure too.

> 🧠 **"Left = Lungs."**

SELECT ALL THAT APPLY

Which interventions reduce the heart's workload in a child with heart failure?

A. Large feedings every 6 hours

B. Small, frequent feedings with rest periods

C. Vasodilators

D. Increasing fluid intake

E. Diuretics

F. Inotropic medications such as digoxin

Swipe for answer →

Answer 37

B, C, E, F

- **Small, frequent feedings with rest periods**
- **Vasodilators**
- **Diuretics**
- **Inotropic medications such as digoxin**

Why: The addendum: reduce workload with diuretics, inotropes, and vasodilators. Small frequent feeds save energy.

> 🧠 **"Dump, Pump, Relax": Diuretics dump fluid, Inotropes help it pump, Vasodilators relax the vessels.**

SINGLE ANSWER

What are the EARLIEST signs of heart failure in an infant?

A. Clubbing

B. Ankle edema

C. Bradycardia and hypertension

D. Tachycardia and tachypnea

Swipe for answer →

Answer 38

D

- **Tachycardia and tachypnea**

Why: The addendum lists tachycardia and tachypnea first. Infants show HF through fast breathing, fast heart rate, and feeding trouble before edema.

> 🧠 **"Fast and Fast": fast heart, fast breathing.**

SINGLE ANSWER

Before giving digoxin to an infant, what should the nurse do?

A. Take the radial pulse for 15 seconds

B. Check the blood pressure only

C. Count the apical pulse for 1 full minute and hold if below the prescribed limit

D. Give it with a full bottle of formula

Swipe for answer →

Answer 39

C

- **Count the apical pulse for 1 full minute and hold if below the prescribed limit**

Why: Digoxin slows the heart. An apical pulse for a full minute is required; hold and notify if below parameters (often under 90–110 in infants).

> 🧠 **"Dig: count a full minute, then Don't go if it's low."**

SINGLE ANSWER

A child receives furosemide and digoxin. Which lab value is the priority to monitor?

A. Glucose

B. Potassium

C. Hemoglobin A1c

D. Calcium

Swipe for answer →

Answer 40

B

- **Potassium**

Why: Furosemide wastes potassium, and low potassium increases the risk of digoxin toxicity.

> 🧠 **"Lasix Loses K → Dig gets Dangerous."**

SELECT ALL THAT APPLY

Which findings are consistent with rheumatic fever?

A. Swollen, tender joints

B. Chorea

C. Strawberry tongue

D. Macular rash on the trunk (erythema marginatum)

E. Peeling of the fingertips

F. Subcutaneous nodules

G. New systolic murmur

Swipe for answer →

Answer 41

A, B, D, F, G

- **Swollen, tender joints**
- **Chorea**
- **Macular rash on the trunk (erythema marginatum)**
- **Subcutaneous nodules**
- **New systolic murmur**

Why: These are the Jones major criteria from the addendum. Strawberry tongue and peeling point to Kawasaki.

> 🧠 **"JONES": Joints, O (♥ carditis), Nodules, Erythema marginatum, Sydenham chorea.**

SINGLE ANSWER

Which lab result shows a recent strep infection in suspected rheumatic fever?

A. Elevated troponin only

B. Positive blood culture for Staph

C. Elevated ASO titer

D. Low platelet count

Swipe for answer →

Answer 42

C

- **Elevated ASO titer**

Why: The addendum: positive ASO titer plus increased ESR and CRP. ASO shows PRIOR strep; it doesn't diagnose RF by itself.

> 🧠 **"ASO = Already Strep Occurred."**

SINGLE ANSWER

The nurse notes chorea in a child with rheumatic fever. Which describes chorea?

A. Firm nodules over tendons

B. Swelling of the large joints

C. Sudden, involuntary, purposeless movements of the limbs

D. A painless rash on the trunk

Swipe for answer →

Answer 43

C

- **Sudden, involuntary, purposeless movements of the limbs**

Why: Sydenham chorea = jerky involuntary movements. It may appear later than other signs.

> 🧠 **"Chorea = Choreography gone wild" (dancing that the child can't control).**

SINGLE ANSWER

What determines the long-term prognosis of rheumatic fever?

A. How much the heart (especially the valves) is involved

B. How many joints were swollen

C. How high the fever got

D. Whether the rash was itchy

Swipe for answer →

Answer 44

A

- **How much the heart (especially the valves) is involved**

Why: Arthritis heals without damage; carditis can leave permanent valve disease (mitral/aortic).

> 🧠 **"Joints heal, the Heart remembers." Rheumatic = Valves.**

SELECT ALL THAT APPLY

Which are part of managing acute rheumatic fever?

A. IVIG as first-line therapy

B. Bed rest during the acute phase

C. Ongoing prophylaxis to prevent recurrence

D. Antibiotics to eliminate group A strep

E. Ear tubes

Swipe for answer →

Answer 45

B, C, D

- **Bed rest during the acute phase**
- **Ongoing prophylaxis to prevent recurrence**
- **Antibiotics to eliminate group A strep**

Why: The addendum: bed rest and antibiotics to eliminate GABHS. Secondary prophylaxis (e.g., benzathine penicillin) prevents recurrence.

> 🧠 **"Kill the strep, Rest the heart, Prevent the repeat."**

SELECT ALL THAT APPLY

Which are EARLY signs of Kawasaki disease?

A. Swollen hands and feet

B. Coronary artery aneurysm

C. High fever that doesn't respond to antipyretics

D. Peeling skin on the fingers

E. Strawberry tongue and red, cracked lips

F. Red eyes (conjunctival injection) without discharge

G. Enlarged cervical lymph nodes

Swipe for answer →

Answer 46

A, C, E, F, G

- **Swollen hands and feet**
- **High fever that doesn't respond to antipyretics**
- **Strawberry tongue and red, cracked lips**
- **Red eyes (conjunctival injection) without discharge**
- **Enlarged cervical lymph nodes**

Why: Peeling (desquamation) and aneurysms are LATE findings on the addendum.

> 🧠 **"CRASH and BURN": Conjunctivitis, Rash, Adenopathy, Strawberry tongue, Hands/feet + BURN (fever for 5+ days).**

SELECT ALL THAT APPLY

Which are LATE findings in Kawasaki disease?

A. Coronary artery aneurysm

B. Onset of high fever

C. Strawberry tongue appearing for the first time

D. Peeling skin (desquamation)

E. Rising platelet count

Swipe for answer →

Answer 47

A, D, E

- **Coronary artery aneurysm**
- **Peeling skin (desquamation)**
- **Rising platelet count**

Why: The addendum's late signs: desquamation, increased platelets, aneurysms. High platelets raise the clot risk in aneurysms.

> 🧠 **"Late = Peel, Platelets, Pouches" (aneurysms are pouches).**

SINGLE ANSWER

What is the main purpose of IV immunoglobulin (IVIG) in Kawasaki disease?

A. To decrease the immune response and lower the risk of coronary aneurysms

B. To close the ductus arteriosus

C. To raise the platelet count

D. To kill the bacteria causing the disease

Swipe for answer →

Answer 48

A

- **To decrease the immune response and lower the risk of coronary aneurysms**

Why: IVIG calms the inflammation; given early (within about 10 days) it reduces coronary damage. NOTE: your addendum lists ibuprofen for inflammation, but your integrated study guide and current AHA guidance use ASPIRIN with IVIG. Ibuprofen can block aspirin's anti-clotting effect. Ask your professor which answer they expect.

> 🧠 **"Kawasaki Crowns the Coronaries." IVIG protects the crowns.**

SINGLE ANSWER

Which follow-up is MOST important after Kawasaki disease?

A. Monthly throat cultures

B. Yearly hearing tests

C. Daily blood glucose checks

D. Serial echocardiograms to check the coronary arteries

Swipe for answer →

Answer 49

D

- **Serial echocardiograms to check the coronary arteries**

Why: The big risk is coronary aneurysm, which echo detects. Most children recover fully, but some need heart surgery.

> 🧠 **"Echo the coronaries."**

SINGLE ANSWER

A child received IVIG for Kawasaki disease. When can the MMR and varicella vaccines be given?

A. 1 month after IVIG

B. Right away

C. Never

D. 11 months after IVIG

Swipe for answer →

Answer 50

D

- **11 months after IVIG**

Why: IVIG's antibodies block the response to live vaccines. Non-live vaccines don't need to wait.

> 🧠 **"2 grams → wait 11 months."**